

(POM)

SANZYME FSH

Sanzyme FSH 75 IU (Urofollitropin Lyophilised Powder for Solution for Injection BP, 75 IU/vial) & Sodium Chloride Injection USP 1ml Ampoule (Sterile 0.9% w/v solution of Sodium Chloride)

Sanzyme FSH 150 IU (Urofollitropin Lyophilised Powder for Solution for Injection BP, 150 IU/vial) & Sodium Chloride Injection USP 1ml Ampoule (Sterile 0.9% w/v solution of Sodium Chloride)

Lyophilized powder, For IM/SC use only

PRESENTATION

Urofollitropin for Injection BP 75 IU/Vial

Each Vial of sterile freeze-dried product contains:

Urofollitropin (FSH) BP.....75 IU
Mannitol USP.....q.s.
Potassium Dihydrogen Phosphate BP.....q.s.
Dipotassium Hydrogen Phosphate BP.....q.s.

Urofollitropin for Injection BP 150 IU/Vial

Each Vial of sterile freeze-dried product contains:

Urofollitropin (FSH) BP.....150 IU
Mannitol USP.....q.s.
Potassium Dihydrogen Phosphate BP.....q.s.
Dipotassium Hydrogen Phosphate BP.....q.s.

Product:

For 75 IU: An almost white or slightly yellowish freeze-dried product sealed in a 2mL USP Type-I clear glass vials with 13mm Grey bromobutyl rubber stoppers and Burgandy colour flip off seals.

For 150 IU: An almost white or slightly yellowish freeze-dried product sealed in a 2mL USP Type-I clear glass vials with 13mm Grey bromobutyl rubber stoppers and Light pink colour flip off seals.

Diluent:

A clear colorless solution. 1mL filled and duly sealed in 2mL USP Type I clear glass ampoule.

After reconstitution:

Clear colorless solution. The product SANZYME FSH 75/150 IU is compatible with the reconstitution solutions i.e. 0.9%w/v sodium chloride injection.

PHARMACOLOGICAL PROPERTIES

Pharmacodynamic:

SANZYME FSH contains purified hormone Urofollitropin, obtained from human menopausal urine having FSH activity of 75 IU/ 150 IU and less than 1 IU/2 IU Luteinizing Hormone activity per vial.

Patients showing normal or high LH levels (WHO Group II), require preparations without LH activity, this character belongs to FSH.

FSH stimulates both the growth and maturation of follicles, it induces secretion of Oestrogens and proliferation of the endometrium"

Pharmacokinetics:

After multiple intramuscular dosing of Urofollitropin, the maximum plasma concentration of Follicle-stimulating hormone occurs about 10 hours after a dose and has an elimination half-life of about 15 or 20 hours respectively.

Because there are no longer ovarian steroids and inhibit to inhibit LH and FSH secretion, gonadotropin secretion is greatly increased, and the levels of FSH are higher than those of LH in postmenopausal women.

FSH and LH are eliminated in two phases. In both phases the biological half-life of FSH is longer than that of LH.

The gonadotropins of either pituitary or placental origin are effective only if given by injection, usually intramuscularly. The rate at which gonadotropins are cleared from plasma is difficult to measure

accurately because of their structural heterogeneity and the background of pulsatile secretion of the SANZYME FSH hormones.

The prolonged circulatory life of these glycoprotein hormone relative to many other peptide hormones is a result of their resistance to metabolic degradation in most tissue beds. Clearance of injected (and presumably of SANZYME FSHous) glycoprotein hormones is by glomerular filtration, followed by degradation in the proximal renal tubule or excretion (unchanged) in the urine. Removal of the sialic acid residues results in their rapid and complete clearance by the hepatic reticuloendothelial system.

Indications:

SANZYME FSH is indicated for stimulation of follicular growth in infertile women. A course of SANZYME FSH is usually followed by human chorionic gonadotrophin (hCG, Pubergen) to induce ovulation.

SANZYME FSH is used as:

(a) For single follicular development in cases such as hypothalamic pituitary dysfunction (WHO Group II classification), include patients with polycystic ovarian disease.

(b) For multiple follicular development (assisted conception techniques); in cases such as tubal occlusion, unexplained infertility and male factor infertility.

DOSAGE

The dose of SANZYME FSH to produce maturation of follicle must be individualized for each patient.

It is recommended that initial dose to any patient should be 75 IU of SANZYME FSH per day administered subcutaneous for at least 7 days followed by HCG 5000 IU to 10000 IU one day after last dose of SANZYME FSH. Courses of treatment should be no longer than 12 days.

If there is evidence of ovulation but no pregnancy, repeat above dosage regimen for at least 2 or more courses before increasing the dose to SANZYME FSH 150 IU per day for 7-12 days. As stated above this dose should be followed by HCG 5000 IU to 10000 IU one day after last dose of SANZYME FSH. If evidence of ovulation is present but pregnancy is not sure repeat the same dose for 2 more courses. Doses larger than this are not routinely recommended.

In-vitro Fertilization:

In-vitro fertilization therapy with SANZYME FSH should be initiated in the early follicular phase (cycle day 2 or 3) at a dose of 150 IU per day until sufficient follicular development is attained. In most cases therapy should not exceed beyond 10 days.

Method of administration:

SANZYME FSH is intended for subcutaneous (SC) & intramuscular (IM) injection after reconstitution with the solvent provided. The powder should be reconstituted immediately prior to use. In order to avoid the injection of large volumes up to 6 vials of the powder may be dissolved in the solvent provided. The solution should not be used if it contains particles or if it is not clear.

Appearance of reconstituted solution:

Clear solution

CONTRAINDICATIONS

SANZYME FSH is contraindicated in women who exhibit:

- High levels of FSH, indicating primary ovarian failure.
 - Uncontrolled thyroid or adrenal dysfunction.
 - An organic intracranial lesion such as pituitary tumor.
 - The presence of any cause of infertility other than anovulation unless they are candidates for in vitro fertilization.
 - Ovarian cysts or enlargement not due to ovarian polycystic ovarian disease.
 - Prior hypersensitivity to Urofollitropin.
 - SANZYME FSH is contraindicated in women who are pregnant.
- There are limited human data on the effects of SNAYME FSH when

administered during pregnancy.

Contraindicated for safety reasons in gynecological hemorrhages of unknown aetiology

WARNINGS AND PRECAUTIONS

Before starting treatment, the couple's infertility should be assessed as appropriate and putative contra-indications for pregnancy evaluated. Adherence to the recommended dosage and monitoring schedules will minimize the possibility of ovarian hyper stimulation syndrome.

Excessive ovarian response to SANZYME FSH treatment does generally not induce significant adverse effects except if hCG is administered for ovulation induction or if pregnancy occurs; ovarian hyper stimulation syndrome occurs usually 1 to 2 weeks following hCG administration and ovulation.

In case of symptoms such as pelvic pain, abdominal distension or ovarian enlargement or if oestrogen assays or ultrasound examinations suggest an excessive oestrogenic response, Folliculin administration should be discontinued and hCG should not be administered and intercourse avoided in order to prevent ovarian hyper stimulation.

Ascites, pericardial effusion, hydrothorax, hemo-concentration, secondary hyperaldosteronism or hypercoagulability might appear. These symptoms should be controlled through appropriate medical measure, including avoidance of unnecessary pelvic examination. In the absence of pregnancy, they usually resolve spontaneously with the onset of the menses.

Interaction with other medicines:

No clinically significant adverse drug/drug or drug/food interactions have been reported during SANZYME FSH therapy. Concomitant use of SANZYME FSH and Clomifene citrate may potentiate the follicular response, whereas concurrent use of GnRH agonist induced pituitary desensitization may increase the dosage of SANZYME FSH needed to elicit an adequate ovarian response.

Pregnancy and lactation:

Should not be given if pregnancy is suspected or to lactating mothers.

Undesirable effects:

Local reactions at the injection site, fever and arthralgias have been reported following Urofollitropins and Menotropins administration.

Gastrointestinal symptoms may occur as well as bloating of the stomach, pelvic pain or sore breasts. Mild to moderate ovarian enlargement, ovarian cysts may be observed.

Severe hyper stimulation syndrome is rare. In rare instances, arterial thromboembolisms have been associated with Menotropin human Chorionic Gonadotrophin therapy.

Pregnancy wastage by miscarriage or abortion is comparable with the rates in women with other fertility problems. Ectopic pregnancy may occur in women with a history of prior tubal disease.

Overdose:

There are no reports of toxic effects occurring as a result of over dosage. However, an ovarian hyper stimulation syndrome cannot be ruled out. Aside from possible ovarian overstimulation and multiple gestations, little is known concerning the consequence of acute over dosage with SANZYME FSH.

Incompatibilities:

This medicinal product must not be mixed with other medicinal products.

Storage Conditions:

Storage condition of the product SANZYME FSH 75 / 150 IU: Store in a refrigerator (2-8°C). Store in the original carton to protect from light.

Storage condition of the diluent (Reconstitution Solution): "Store in a refrigerator (2-8°C). Do not freeze. Store in the original carton to protect

from light."

Storage condition of the diluent (0.9% w/v Sodium chloride Injection): After Reconstitution, solution should be used immediately and discard any unused solution.

INSTRUCTIONS OF USAGE

Reconstitute the contents of vial containing SANZYME FSH in 1ml of Sodium Chloride Injection BP and administer subcutaneous immediately.

Dosage forms and packing available:

Box containing 1 vial of sterile freeze-dried Urofollitropin for Injection + 1 Ampoule of sodium chloride injection USP (0.9%w/v).

Shelf Life:

Shelf life of the product SANZYME FSH 75 /150 IU: 36 months when stored at a temperatures between 2–8°C.

Shelf life of the diluent (Reconstitution Solution): 60 months when stored at a temperatures between 2–8°C.

Shelf life after reconstitution of the product: After Reconstitution, solution should be used immediately and discard any unused solution.

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Product Registration Holder:

HACZEN SDN BHD
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